A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

Dr Lucy Haymes
Ontario Family Practice

Dear Dr Haymes,

Re: Ms Tayla Kane
DOB: 25/10/1997 Phone: 0429 411 675
Address: 10/939 Karadoc Avenue, IRYMPLE, VIC, 3498
Email: taylajkane@gmail.com

Summary

- **Stable mild Crohn's disease in the setting of pregnancy.**
- **Symptomatic reflux for management with omeprazole and nizatidine.**
- **Post-partum review in six months with repeat inflammatory markers.**
- **No action required by you.**

I had the pleasure of reviewing Tayla in a telehealth consultation. She is currently pregnant with her third child and she reports that her obstetrician is pleased with the progress of the pregnancy.

From a luminal perspective, Tayla's Crohn's disease remains on the milder end of the spectrum. Her recent blood results are satisfactory. We discussed the long-term management of her condition, particularly the safety of various Crohn's therapies during pregnancy and breastfeeding. While I have reassured her that biological therapies are considered safe should they be required in the future, her current stable clinical state does not necessitate any escalation of treatment at this juncture.

Tayla has been experiencing significant reflux symptoms, for which she has been using Rennie. I have suggested that Gaviscon liquid may be a more effective over-the-counter option. Furthermore, I have provided her with prescriptions for omeprazole and nizatidine. I have advised her to take the omeprazole regularly in the morning before breakfast for maximal efficacy, while the nizatidine may be used on an as-needed basis. She will confirm the use of these medications with her obstetrician, although both are well-established as safe for use in pregnancy.

Impression and Plan I intend to review Tayla in approximately six months' time, which should allow her a sufficient recovery period post-delivery. At that stage, I have asked her to complete repeat blood tests and a faecal calprotectin to objectively assess the degree of underlying inflammation. If her symptoms remain mild and her markers are low, we may continue with a monitoring strategy. However, if there is evidence of significant activity, we will formalise a plan for regular maintenance therapy to keep her Crohn's in remission. I will arrange for this follow-up to occur through my clinic in the public hospital system in early next year.

Yours sincerely,

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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CC: Dr Tom Gleeson

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